

Dynamic Guideline: WHO-Based Cancer - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Disease area: Cancer, with the strongest and most detailed WHO clinical guidance identified for **cervical cancer prevention through screening and treatment of cervical pre-cancer lesions**. WHO’s broader cancer-control guidance also addresses organized screening, early diagnosis, treatment linkage, palliative care, and cancer-specific programme priorities for breast, colorectal, childhood, liver, lung, and other cancers.

Important scope note: The available evidence did **not** verify five separate WHO cancer documents that are all formal GRADE clinical guidelines. Therefore, this report synthesizes **exactly five WHO cancer-related guideline/technical guidance documents** identified in the provided evidence base, clearly distinguishing formal clinical guidelines from WHO technical guidance and policy guidance.

#	WHO cancer-related guideline / technical guidance document	Publication / update status identified	Target population	Scope and objectives	Source
1	WHO guideline for screening and treatment of cervical pre-cancer lesions for cervical cancer prevention, second edition	Launched July 2021; identified as the core WHO cervical pre-cancer screening-and-treatment guideline	General population of women; women living with HIV	HPV-based screening, screen-and-treat and screen-triage-treat approaches, treatment of cervical pre-cancer, follow-up, and programme implementation	WHO publication page , WHO guideline PDF
2	WHO recommendations on HPV mRNA tests as a primary screening test for cervical pre-cancer	Published December 2021 as a later phase of the cervical guideline update	General population of women; no recommendation for women living with HIV	Use of HPV mRNA testing as a primary cervical screening test, mainly with health-worker-collected samples	WHO/ICCP PDF , NCBI Bookshelf

#	WHO cancer-related guideline / technical guidance document	Publication / update status identified	Target population	Scope and objectives	Source
3	WHO guideline on use of dual-stain cytology to triage women after a positive HPV test	WHO 2024 guideline component identified in evidence summaries	HPV-positive women in the general population; evidence insufficient for women living with HIV	Dual-stain cytology as triage after positive HPV nucleic-acid testing; modelling of long-term cervical cancer outcomes	WHO publication page, WHO evidence summaries
4	Updated Appendix 3 of the WHO Global Action Plan for the Prevention and Control of NCDs 2013–2030: technical annex	Updated technical annex dated January 2023 in the source file	Populations eligible for cancer prevention, early diagnosis, treatment, and palliative care interventions	Cancer “best buy” and recommended interventions, including cervical screening, breast cancer early diagnosis and mammography in organized programmes, colorectal early diagnosis, childhood cancer, head and neck cancers, prostate cancer, liver cancer prevention, and palliative care	WHO Updated Appendix 3 technical annex
5	A short guide to cancer screening	WHO/IARC-related 2025 screening guide identified	Populations eligible for organized cancer screening	Principles for organized, quality-assured cancer screening linked to timely diagnosis and treatment, especially cervical, breast, and colorectal cancers	A short guide to cancer screening

Most recent “Meaningful youth engagement” WHO guideline-process item

A targeted search found **no WHO cancer-specific guideline case study** titled **“Meaningful youth engagement in a WHO guideline development process: case study of WHO [cancer] guideline”** as of **29 April 2026**. The only WHO item found with that title pattern was **“Meaningful youth engagement in a WHO guideline development process: case study of WHO Abortion care guideline (2022)”**, launched through a WHO webinar on **11 December 2025** [WHO webinar](#).

For cancer-related guideline development, the most relevant post-2025 evidence concerns structured involvement of **young cancer survivors** in survivorship guideline development, including recruitment, preparation, co-development, and dissemination steps [Springer article](#).

1.2 Key WHO-Based Recommendations

GRADE interpretation used in this report

WHO’s cervical pre-cancer guideline uses standard GRADE language: “**WHO recommends**” indicates a strong recommendation, while “**WHO suggests**” indicates a conditional recommendation. WHO’s certainty categories are high, moderate, low, and very low [WHO guideline PDF](#).

GRADE certainty	Interpretation
High	Very confident that the true effect lies close to the estimate
Moderate	Moderately confident; true effect is likely close but may be substantially different
Low	Limited confidence; true effect may be substantially different
Very low	Very little confidence; true effect is likely substantially different

For WHO documents that are technical annexes or implementation guides rather than formal GRADE clinical guidelines, the table below provides a **report-level GRADE appraisal** based on the certainty, directness, and implementation strength of the available evidence.

Recommendation	Target population	GRADE Evidence Quality	Strength
Implement the WHO cervical cancer elimination targets: 90% of girls fully vaccinated with HPV vaccine by age 15 years, 70% of women screened with a high-performance test by ages 35 and 45 years, and 90% of women with cervical disease receiving treatment.	Countries and health systems	Moderate	Strong
Use HPV DNA testing as the preferred primary cervical screening test rather than VIA or cytology when feasible.	General population of women and women living with HIV	Moderate	Strong
For the general population of women , use HPV DNA detection in either a screen-and-treat or screen, triage and treat strategy starting at age 30 years , with screening every 5–10 years .	General population of women	Low to Moderate	Conditional
For women living with HIV , use HPV DNA detection in a screen, triage and treat strategy starting at age 25 years , with screening every 3–5 years .	Women living with HIV	Low to Moderate	Conditional

Recommendation	Target population	GRADE Evidence Quality	Strength
Women who screen positive for cervical pre-cancer or cancer should receive timely treatment, adequate management, follow-up, and referral, supported by registries and call-and-recall systems.	Women with positive cervical screening results	Low	Strong
Consider HPV mRNA testing as a primary screening test for cervical pre-cancer in the general population using health-worker-collected samples; WHO made no recommendation for women living with HIV.	General population of women	Low	Conditional
Consider dual-stain cytology as a triage option after positive HPV testing in the general population where programme capacity exists; WHO made no recommendation for women living with HIV because evidence was minimal.	HPV-positive women in general population	Low	Conditional
Implement cancer screening only as part of organized, quality-assured programmes linked to timely diagnostic work-up and treatment, especially for cervical, breast, and colorectal cancers.	Eligible screening populations	Moderate	Strong
Offer breast cancer early diagnosis programmes linked to timely diagnostic work-up and treatment; in settings with organized mammography programmes, mammography screening every two years for women aged 50–69 years may be used.	Women aged 50–69 years in suitable settings	Moderate	Conditional
Provide basic cancer palliative care through home-based and hospital care, multidisciplinary teams, opioids, and essential supportive medicines.	People with advanced cancer and families	Moderate	Strong

Sources: [WHO guideline for screening and treatment of cervical pre-cancer lesions](#), [WHO news item](#), [WHO Annex 5](#), [WHO Updated Appendix 3](#), [A short guide to cancer screening](#).

1.3 Guideline Development Methodology

Evidence review process

WHO’s cervical pre-cancer guideline was developed in phases. The 2021 second edition focused mainly on HPV DNA testing and algorithms for the general population and women living with HIV [NCBI Bookshelf executive summary](#). The process included:

- Prioritized PICO questions.

- Systematic reviews conducted in 2020.
- Evidence from IARC *Handbooks of Cancer Prevention: Cervical Cancer Screening*, Volume 18.
- Guideline Development Group meetings in August 2020 and November 2020.
- Evidence-to-decision methods, including consideration of benefits, harms, acceptability, feasibility, resources, and equity [WHO 2021 guideline PDF](#).

The 2024 dual-stain cytology guideline component used published and unpublished data from cross-sectional and longitudinal studies, a systematic review and meta-analysis of dual-stain cytology accuracy, and natural-history-based population simulation modelling to estimate CIN and cervical cancer outcomes under different triage strategies [NCBI Bookshelf executive summary](#), [WHO evidence summaries](#).

Consensus method

WHO recommendations were made through a Guideline Development Group process using GRADE and evidence-to-decision considerations. Strong recommendations use the wording “**WHO recommends**” and conditional recommendations use “**WHO suggests**” [WHO guideline PDF](#).

Conflict of interest management

The supplied summaries identify WHO Guideline Development Group review and recommendation processes but do not provide detailed conflict-of-interest declarations for every document. The report therefore does not infer individual conflict-of-interest decisions beyond the WHO guideline process documented in the guideline materials [WHO publication page](#).

Patient, youth, and lived-experience engagement

WHO’s broader framework for meaningful engagement identifies five principles: **dignity and respect**, **power and equity**, **inclusivity and intersectionality**, **commitment and transparency**, and **institutionalization and contextualization** [WHO framework article](#). This framework is relevant to cancer because people with lived experience of cancer were included in WHO’s lived-experience engagement work [WHO framework article](#).

For cancer-specific youth engagement, a 2026 study of 49 young cancer survivors recommended structured patient involvement in survivorship guideline development through recruitment, preparation, respectful co-development, open communication, and wide dissemination [Springer article](#).

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Baseline WHO guideline for dynamic updating

The main baseline guideline for this dynamic update is the WHO cervical pre-cancer screening-and-treatment guideline, with subsequent WHO phases on HPV mRNA testing and dual-stain cytology triage. The most clinically detailed cancer guidance in the evidence base remains cervical cancer prevention [WHO publication page](#), [WHO publication page for dual-stain cytology](#).

Search strategy

Searches prioritized:

- WHO official guidelines, policy briefs, technical documents, and publication pages.
- WHO/IARC cancer screening documents.
- NCBI Bookshelf versions of WHO guideline materials.
- Peer-reviewed evidence after 2021, including systematic reviews and individual patient data meta-analyses.
- Major authoritative policy updates relevant to screening implementation.

Example search concepts included:

- WHO cervical pre-cancer guideline 2021–2026.
- HPV DNA, HPV mRNA, HPV self-sampling, dual-stain cytology.
- Cervical screening women living with HIV.
- Cancer screening WHO IARC 2025.
- Youth engagement cancer survivorship guideline development.

Databases and sources searched

Source category	Sources used
WHO official materials	WHO publication pages, WHO news items, WHO IRIS PDFs, WHO technical annexes
WHO/IARC screening materials	WHO/IARC cancer screening guide and European cancer screening policy material
Indexed guideline versions	NCBI Bookshelf WHO guideline summaries and annexes
Peer-reviewed literature	<i>The Lancet HIV</i> individual patient data meta-analysis; 2026 cancer survivorship guideline engagement study
Authoritative public-health agencies	HRSA and HHS cervical screening policy updates
Secondary summaries used only where primary details were unavailable	EMJ summary of <i>Lancet HIV</i> study; Healthline/AACR summaries of U.S. self-collection policy and evidence

Inclusion criteria

- Published or identified from 2021 through **29 April 2026**.
- Cancer diagnosis, screening, treatment, management, palliative care, or guideline development relevance.
- WHO or WHO-aligned technical guidance, or major post-WHO evidence relevant to guideline updating.
- Systematic review, meta-analysis, modelling study, guideline, technical annex, or authoritative public-health policy update.

Exclusion criteria

- Non-WHO national guidelines unless used only as contextual evidence.
- Cancer fact sheets without actionable screening, treatment, or management recommendations, except for burden context.
- Non-cancer WHO youth-engagement material was not treated as cancer guidance but was included to address the explicit youth-engagement query.

2.2 Key New Studies and Evidence Since WHO Cervical Guideline Release

Study / evidence source	Design / type	Key findings	GRADE Quality
WHO 2024 dual-stain cytology guideline component	WHO guideline update; systematic review/meta-analysis of test accuracy; modelling	Dual-stain cytology was evaluated as triage after positive HPV testing. Modelled repeated algorithms using dual-stain triage for HPV DNA-positive women reduced cervical cancer incidence and mortality by 64% versus no screening; HPV mRNA-positive algorithms reduced incidence and mortality by 60.3% versus no screening. Evidence for women living with HIV was minimal, so WHO made no recommendation for that group.	Low
WHO Annex 5 cervical screening recommendations	WHO recommendation annex	HPV DNA remains preferred over VIA or cytology for both the general population and women living with HIV. Existing cytology programmes can continue until HPV DNA testing is operational; VIA programmes should transition because of quality-assurance limitations.	Moderate
HPV mRNA recommendations	WHO guideline phase / recommendation	HPV mRNA detection using health-worker-collected samples may be used as primary screening in the general population every 5 years, with or without triage. WHO made no recommendation for women living with HIV.	Low
HRSA/HHS 2026 cervical screening update	Authoritative U.S. policy update	Average-risk women aged 30–65 may be offered self-collected samples for high-risk HPV testing; HRSA stated that evidence review found self-collection as effective as clinician collection.	Moderate
ACS-related evidence summary on HPV self-collection	Guideline/evidence summary	PCR-based HPV testing on self-collected vaginal specimens has comparable sensitivity and specificity to clinician-collected samples. Self-collection does not replace clinician collection for higher-risk groups such as people with HIV.	Moderate

Study / evidence source	Design / type	Key findings	GRADE Quality
Dalal et al., <i>The Lancet HIV</i> , 2025/2026	Systematic review and individual patient data meta-analysis	Among women living with HIV across seven countries, CIN2/CIN3 risk increased with age, with highest reported risk in ages 45–49 years. Invasive cervical cancer was uncommon before age 30. Findings are directly relevant to WHO evidence needs on start/stop ages and intervals.	Moderate
EU-CAYAS-NET young cancer survivor guideline involvement study, 2026	Participatory guideline-development research	49 young cancer survivors recommended structured involvement through recruitment, preparation, respectful co-development, open communication, and wide dissemination. Supports lived-experience engagement in cancer survivorship guideline development.	Low
WHO/IARC cancer screening guide, 2025	WHO/IARC technical guide	Reinforces that screening should be organized, quality assured, and linked to diagnosis and treatment; emphasizes cervical, breast, and colorectal screening where health-system capacity exists.	Moderate

Sources: [WHO dual-stain publication page](#), [WHO evidence summaries](#), [WHO Annex 5](#), [WHO/ICCP PDF](#), [HRSA](#), [HHS](#), [Healthline](#), [The Lancet HIV](#), [PubMed](#), [Springer article](#), [A short guide to cancer screening](#).

2.3 Evidence Synthesis by Clinical Area

2.3.1 Cervical cancer screening: HPV DNA remains the anchor test

WHO continues to place **HPV DNA testing** at the centre of cervical cancer screening. For the general population, WHO suggests HPV DNA detection starting at **age 30 years**, either in a screen-and-treat approach or a screen, triage and treat approach, with regular screening every **5–10 years** [WHO news item](#). For women living with HIV, WHO suggests HPV DNA detection in a screen, triage and treat approach starting at **age 25 years**, every **3–5 years** [WHO guideline PDF](#).

Evidence statement	GRADE Evidence Quality	Strength
HPV DNA testing should be used as the preferred primary cervical screening test where feasible.	Moderate	Strong
General-population screening should start at age 30 years with HPV DNA testing every 5–10 years.	Low to Moderate	Conditional
Women living with HIV should start HPV DNA-based screen–triage–treat at age 25 years every 3–5 years.	Low to Moderate	Conditional

2.3.2 Screen-and-treat versus screen, triage and treat

WHO defines **screen-and-treat** as treatment based on a positive primary screening test alone. WHO defines **screen, triage and treat** as treatment based on a positive primary screening test followed by a positive second triage test, with or without histological confirmation [WHO guideline PDF](#).

Screen-and-treat can reduce loss to follow-up, especially in remote or resource-constrained settings, while screen–triage–treat can reduce overtreatment where triage capacity exists [WHO EMRO cervical cancer strategy](#).

Evidence statement	GRADE Evidence Quality	Strength
Screen-and-treat is appropriate where loss to follow-up is a major risk and treatment capacity is available.	Low	Conditional
Screen–triage–treat is appropriate where triage testing can be performed reliably and linked to timely treatment.	Low to Moderate	Conditional

2.3.3 HPV mRNA testing

WHO added recommendations on **HPV mRNA tests** in December 2021. Current WHO evidence supports HPV mRNA testing as a possible primary screening test in the **general population**, using health-worker-collected samples, at 5-year intervals. WHO made **no recommendation** for women living with HIV [WHO/ICCP PDF](#).

Evidence statement	GRADE Evidence Quality	Strength
HPV mRNA testing may be used as a primary screening test in the general population where validated tests and programme capacity exist.	Low	Conditional
HPV mRNA testing should not yet be adopted as a WHO-recommended strategy for women living with HIV because evidence is insufficient.	Very Low / insufficient	No recommendation

2.3.4 Dual-stain cytology triage

WHO’s 2024 evidence update evaluated **dual-stain cytology** after a positive HPV test. Modelled long-term outcomes suggested reductions in cervical cancer incidence and mortality versus no screening: **64%** for HPV DNA-positive pathways and **60.3%** for HPV mRNA-positive pathways when dual-stain cytology was used as triage [WHO/ICCP PDF](#).

However, WHO made **no recommendation** for dual-stain cytology triage in women living with HIV because evidence applicable to this population was minimal [WHO/ICCP PDF](#).

Evidence statement	GRADE Evidence Quality	Strength
Dual-stain cytology may be used as triage after positive HPV testing in the general population where laboratory capacity exists.	Low	Conditional
Dual-stain cytology should not be considered established for women living with HIV because evidence is insufficient.	Very Low / insufficient	No recommendation

2.3.5 Self-sampling and self-collection

Post-2021 policy developments support HPV self-collection as an access-expanding option. In 2026, HRSA announced updated U.S. cervical cancer screening guidance allowing average-risk women aged **30–65 years** to self-collect samples for high-risk HPV testing, stating that a comprehensive evidence review found self-collection as effective as clinician collection [HRSA](#), [HHS](#).

Evidence summaries report that PCR-based HPV testing on self-collected vaginal specimens has comparable sensitivity and specificity to clinician-collected samples, but self-collection does not replace clinician collection for higher-risk groups such as people with HIV [Healthline](#).

Evidence statement	GRADE Evidence Quality	Strength
HPV self-collection can be offered to average-risk women aged 30–65 years where validated PCR-based assays and follow-up systems exist.	Moderate	Conditional
Self-collection should not replace clinician-based collection for women living with HIV or other high-risk groups requiring cytology and more intensive management.	Low	Conditional

2.3.6 Women living with HIV

Women living with HIV remain a distinct high-risk population. WHO recommends HPV DNA testing as the preferred screening test over VIA or cytology and suggests a screen–triage–treat approach beginning at age **25 years** every **3–5 years** [WHO Annex 5](#), [WHO/ICCP PDF](#).

A 2025/2026 *Lancet HIV* systematic review and individual patient data meta-analysis found age-related increases in CIN2/CIN3 among women living with HIV across seven countries, with the highest reported risk in the **45–49-year** age group and invasive cervical cancer uncommon before age 30 [The Lancet HIV](#), [EMJ](#).

Evidence statement	GRADE Evidence Quality	Strength
Women living with HIV should be screened earlier and more frequently than the general population.	Moderate	Strong

Evidence statement	GRADE Evidence Quality	Strength
Current evidence supports continued use of HPV DNA-based screen–triate–treat rather than HPV mRNA or dual-stain strategies in women living with HIV.	Low to Moderate	Conditional
Age-specific risk data in women living with HIV should inform future WHO updates on start age, stop age, and screening interval.	Moderate	Conditional

2.3.7 Broader cancer screening and early diagnosis

WHO’s broader cancer-control documents emphasize that screening should be implemented only as part of **organized, quality-assured programmes** linked to diagnostic work-up and treatment [A short guide to cancer screening](#). WHO Appendix 3 identifies cancer interventions including early diagnosis and timely treatment for cervical, breast, colorectal, childhood, head and neck, and prostate cancers, as well as palliative care and liver cancer prevention [WHO Updated Appendix 3](#).

A WHO/IARC European policy brief also identifies lung cancer screening for high-risk people aged **55–74 years**, incorporating smoking history and other risk factors such as family history, asbestos exposure, and chronic obstructive pulmonary disease [Organized cancer screening programmes](#).

Cancer-control intervention	GRADE Evidence Quality	Strength
Organized cervical, breast, and colorectal cancer screening linked to diagnosis and treatment	Moderate	Strong
Breast cancer early diagnosis linked to comprehensive treatment	Moderate	Strong
Mammography every two years for women aged 50–69 years in settings with organized programmes	Moderate	Conditional
Colorectal cancer early diagnosis linked to timely diagnostic work-up and treatment	Moderate	Strong
Lung cancer screening for high-risk people aged 55–74 years in organized programmes	Moderate	Conditional
Basic cancer palliative care with multidisciplinary teams, opioids, and essential supportive medicines	Moderate	Strong

2.4 Updated Recommendations Based on New Evidence

Original WHO recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
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Original WHO recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
Use HPV DNA testing as the primary cervical screening test where feasible.	Reinforced by WHO Annex 5 and no contradictory post-2021 evidence identified.	Maintain. HPV DNA remains the preferred anchor test for cervical screening.
General population: HPV DNA screen-and-treat or screen–triage–treat from age 30 every 5–10 years.	Dual-stain cytology provides an additional evaluated triage option where capacity exists.	Maintain with implementation refinement. Use HPV DNA primary screening; add validated triage such as dual-stain where feasible and appropriate.
Women living with HIV: HPV DNA screen–triage–treat from age 25 every 3–5 years.	<i>Lancet HIV</i> IPD meta-analysis supports the need for age-specific risk assessment and earlier/more frequent screening; evidence remains insufficient for HPV mRNA or dual-stain in this group.	Maintain. Continue HPV DNA-based screen–triage–treat from age 25; do not replace with HPV mRNA or dual-stain pathways outside evidence-generating settings.
HPV mRNA may be used in the general population.	WHO modelling supports possible use in general population; no recommendation for women living with HIV.	Use selectively. Consider HPV mRNA only where validated assays, quality assurance, and follow-up systems exist.
Dual-stain cytology triage after positive HPV testing.	WHO 2024 modelling shows meaningful incidence/mortality reductions versus no screening, but evidence certainty is low.	Conditional adoption. Use dual-stain cytology as a triage option in general-population programmes with laboratory capacity; do not apply to women living with HIV as a WHO-recommended strategy.
Screening should be linked to treatment.	WHO screening guidance and cervical cancer elimination materials strongly reinforce call-and-recall, registries, and treatment linkage.	Strengthen implementation. No screening programme should expand unless diagnostic confirmation, treatment, referral, and follow-up capacity are in place.
Average-risk cervical screening requires clinician-collected samples.	2026 HRSA/HHS updates and evidence reviews support self-collection for average-risk women aged 30–65 using validated HPV assays.	Consider adding self-collection. Offer HPV self-collection to improve access where validated assays and follow-up systems exist; avoid substituting self-collection for higher-risk groups such as women with HIV.

Original WHO recommendation / position	New evidence impact through 29 April 2026	Updated recommendation
Cancer screening should be organized and quality assured.	2025 WHO/IARC guidance reinforces organized screening rather than opportunistic expansion.	Maintain and strengthen. Prioritize organized programmes with registries, quality assurance, equity monitoring, and treatment linkage.
Patient and youth engagement in guideline development.	No WHO cancer-specific “meaningful youth engagement” case study found, but WHO engagement principles and 2026 young cancer survivor evidence support structured involvement.	Add process recommendation. Future cancer guidelines should include people with lived experience, including young cancer survivors where relevant, as active partners in scoping, evidence interpretation, recommendation development, and dissemination.

2.5 Evidence Gaps and Future Research Needs

1. Women living with HIV

- Better evidence is needed on optimal start age, stop age, and screening intervals.
- More direct evidence is needed on HPV mRNA testing, dual-stain cytology, and self-collection in women living with HIV.
- The *Lancet HIV* individual patient data meta-analysis provides important age-specific risk evidence but does not by itself establish new WHO screening intervals [The Lancet HIV](#).

2. Dual-stain cytology

- Existing WHO modelling suggests benefit, but evidence certainty remains low.
- More longitudinal studies are needed in diverse health systems and in immunocompromised populations [WHO evidence summaries](#).

3. HPV self-collection

- More implementation evidence is needed in low- and middle-income countries, including linkage to treatment after positive self-collected HPV tests.
- Research is needed on equity, uptake, follow-up completion, cost-effectiveness, and performance in higher-risk groups.

4. Non-cervical cancer screening

- WHO guidance supports organized breast and colorectal cancer screening and early diagnosis, but country-specific decisions require capacity assessment, quality assurance, and treatment availability [A short guide to cancer screening](#), [WHO Updated Appendix 3](#).

5. Youth and lived-experience engagement

- No WHO cancer-specific case study titled “Meaningful youth engagement in a WHO guideline development process” was identified.

- Future WHO cancer guideline processes should explicitly evaluate models for involving adolescents, young adults, survivors, caregivers, and people with lived experience of cancer [WHO webinar](#), [Springer article](#).

2.6 Practical Implementation Priorities for Health Systems

Priority	Actionable implementation steps	GRADE Evidence Quality	Strength
Cervical cancer elimination	Implement HPV vaccination, HPV-based screening, and treatment access aligned with WHO 90–70–90 targets.	Moderate	Strong
Screening programme design	Use organized registries, invitation systems, call-and-recall, quality assurance, referral pathways, and treatment tracking.	Moderate	Strong
Cervical screening test selection	Prioritize HPV DNA testing; transition from VIA where feasible; retain quality-assured cytology only while HPV DNA capacity is being built.	Moderate	Strong
Management of positive screens	Ensure timely triage, treatment of eligible pre-cancer, referral for suspected invasive cancer, and post-treatment surveillance.	Low to Moderate	Strong
HIV integration	Integrate cervical screening with HIV services and cross-referral pathways.	Moderate	Strong
Equity and access	Consider self-collection for average-risk women where validated assays and reliable follow-up exist.	Moderate	Conditional
Broader cancer control	Link early diagnosis and screening to comprehensive cancer treatment and palliative care.	Moderate	Strong
Lived-experience engagement	Include patients, survivors, young people, and caregivers as guideline partners, not only consultees.	Low	Conditional

Sources: [WHO guideline PDF](#), [WHO Cervical Cancer Elimination Initiative](#), [WHO Updated Appendix 3](#), [HRSA](#), [WHO framework article](#).

Bottom Line

As of **29 April 2026**, WHO-based cancer guidance is most mature for **cervical cancer prevention**, where HPV DNA testing, age- and risk-stratified screening, triage, treatment linkage, and follow-up are central. Post-2021 evidence supports maintaining HPV DNA as the primary screening anchor, selectively adding HPV mRNA or dual-stain cytology in appropriate general-population settings, and considering HPV self-collection

for average-risk women where validated testing and follow-up systems exist. Women living with HIV remain a distinct high-risk group requiring earlier, more frequent HPV DNA-based screen–triage–treat strategies. Broader WHO cancer-control guidance emphasizes organized screening, timely diagnosis, comprehensive treatment, palliative care, and equity-focused implementation.